INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Gemcitabine and cisplatin · lung cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00281
Version 7

Every 3 weeks: gemcitabine and cisplatin on Day 1, then gemcitabine alone on Day 8. Usually 4–6 cycles, with a review before each dose.

Treatment stages

01 **Chemotherapy course**
Usually 4–6 cycles

02 **Response and next-plan review**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Gemcitabine and cisplatin · lung cancer · National Cancer Control Programme (NCCP) 00281

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology team immediately for fever or suddenly feeling unwell, shaking chills, new breathlessness, chest pain, unusual bleeding, very little urine, severe rash or blistering, or vomiting that stops you keeping fluids down. Use your unit's temperature threshold and emergency instructions. Do not wait until the next visit.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

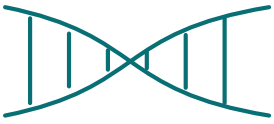
Three medicines. Different jobs.

These two chemotherapy medicines aim to slow or shrink advanced non-small-cell lung cancer and help control symptoms. Benefit is not guaranteed. Your team explains the aim for your own situation.

CHEMOTHERAPY

Gemcitabine

Antimetabolite chemotherapy



Interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) building

Gemcitabine interferes with making DNA, which dividing cancer cells need. Healthy blood-forming cells and other tissues can also be affected.

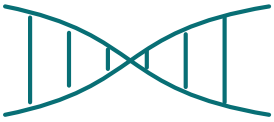
DNA building → Replication disrupted → Dividing cells affected

What to watch for

Report fever, unusual bleeding, breathlessness, rash, swelling or reduced urine promptly.

PLATINUM CHEMOTHERAPY

Cisplatin



Damages DNA through platinum links

Cisplatin forms links in DNA that interfere with copying and repair. This helps stop cancer cells dividing, but healthy tissues may also be affected. Kidney protection with fluids and monitoring is central to cisplatin treatment.

Platinum reaches cells → DNA links form → Cell division disrupted

What to watch for

Report ringing in the ears, hearing changes, numbness or loss of function. Tell the team if you cannot drink, are passing less urine or become swollen or breathless during fluids.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment

Which plan is this?

This guide covers standalone chemotherapy for advanced non-small-cell lung cancer (NSCLC). If nivolumab is part of your treatment before surgery, use the separate 00849 guide for gemcitabine/cisplatin; that is a different three-cycle plan.

How the medicines work

Gemcitabine interferes with the DNA building blocks a dividing cell needs. Platinum makes links within DNA that disrupt copying and repair. Healthy blood-forming cells and other tissues can also be affected.

Benefits and alternatives

A randomized study in previously untreated advanced lung cancer compared gemcitabine with cisplatin against gemcitabine with carboplatin. Both approaches showed anticancer activity. Treatment choice also depends on kidney function, hearing, nerves, blood counts and your preferences. Ask about alternative systemic treatments, clinical trials, symptom-focused care and the choice not to proceed.

Why this combination may be offered

This older study used different doses from the current NCCP schedules and predates modern biomarker-directed and immune treatments. Its averages cannot predict your outcome or establish an individual survival benefit.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Gemcitabine and cisplatin · lung cancer · NCCP 00281

Treatment → review → next steps

Treatment stages

01 **Chemotherapy course**
Usually 4–6 cycles

02 **Response and next-plan review**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

USUALLY 4–6 CYCLES

Chemotherapy course

1 2 3 4 5 if planned 6 if planned

A cycle lasts 21 days (3 weeks). Day 1: gemcitabine and cisplatin. Day 8: gemcitabine alone after separate blood tests and review. Days 9–21: no scheduled chemotherapy. The usual planned course is 4–6 cycles, with earlier changes for progression or unacceptable effects.

INDIVIDUAL PLAN

Response and next-plan review

Review

Your team weighs scan results, symptoms and side effects before deciding to continue, stop or change treatment. No automatic immunotherapy, radiotherapy or surgery phase is included.

Your team confirms each stage and discusses any changes with you.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and checks

Relative sequence; exact visits and tests are prescribed individually.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00281 v7 · official regimen

THE WHOLE TREATMENT JOURNEY

Gemcitabine and cisplatin · lung cancer: treatment and checks

Advanced non-small-cell lung cancer · standalone chemotherapy

A cycle lasts 21 days (3 weeks). Day 1: gemcitabine and cisplatin. Day 8: gemcitabine alone after separate blood tests and review. Days 9–21: no scheduled chemotherapy. The usual planned course is 4–6 cycles, with earlier changes for progression or unacceptable effects.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Day 1	Day 8	At home	Next-cycle review	End-of-course review
Treatment NCCP 00281	Confirm both drugs	Gemcitabine + cisplatin	Gemcitabine only	No routine dose	Repeat if prescribed	Agree next plan
Blood tests NCCP 00281	full blood count (FBC), renal/liver	FBC, renal/liver	FBC, creatinine	Extra tests if needed	Repeat safety tests	Follow unresolved changes
Symptoms and organs NCCP 00281	Fitness, hearing, nerves	Review new symptoms	Review new symptoms	Call promptly when unwell	Hearing/ nerve assessment	Follow lasting effects
Support and response NCCP 00281	Consent and contact plan	Day 1 anti-sickness plan	Day 8 support plan	Take prescribed support	Review benefit and tolerance	Consultant-directed imaging

What each check is for

1 Prepare

Before first dose

Confirm a suitable individual plan

Ask / act: Confirm treatment aim, consent, blood tests and contacts.

2 Each visit

Days 1 and 8

Dose clearance can change

Ask / act: Review symptoms and the correct treatment-day rules.

3

Progress review

As directed by the consultant

Assess benefit as well as toxicity

Ask / **act:** Agree scan timing and next treatment decision.

4

End of course

Usually after 4–6 cycles or earlier if needed

Plan continuing care

Ask / **act:** Agree ongoing support, next treatment and contacts.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before the visit

Confirm blood tests, transport, medicine instructions and contact numbers.

STEP 2

Time in the unit

Gemcitabine runs through a drip over 30 minutes on Days 1 and 8. Cisplatin runs over 60 minutes on Day 1, with fluids and electrolytes before and afterwards according to your prescribed plan. Blood tests, pharmacy preparation, support medicines and review add to the visit time.

STEP 3

Access

Suitable intravenous (IV) access is needed. This plan does not use a routine take-home infusion pump.

STEP 4

Home support

Follow one written anti-sickness prescription, including any tablets for home and rescue treatment. Day 8 gemcitabine has a separate lower-risk anti-sickness plan. Cisplatin needs prescribed IV hydration, electrolyte replacement and kidney, hearing and nerve checks. Follow your own fluid advice; tell the team if you have heart failure or a fluid restriction.

STEP 5

Next appointment

Confirm the next Day 1 or Day 8 visit and any response scan before leaving.

Your visits

Day 8 is a separate decision

Blood count, kidney and liver tests before treatment and on Day 1; blood count and creatinine again before Day 8. Symptoms are reviewed before each dose. Hearing tests are arranged when needed. Scans follow your consultant’s individual plan, not a fixed universal interval.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Gemcitabine and cisplatin · lung cancer · NCCP 00281

Your 21-day cycle.

Timing is a general guide; symptoms may start or stop at other times.

Treatment or check	Timing in this view
Both chemotherapy medicines	 **Day 1** After blood tests and review
Gemcitabine alone	 **Day 8** Separate blood tests and review
Time at home	 **Days 9–21** Effects vary; report concerns without waiting for the next cycle.

Relative cycle days, not guaranteed symptom or recovery dates.

The next cycle starts 21 days after this one, if your team confirms it.

> Call your oncology team immediately for fever or suddenly feeling unwell, shaking chills, new breathlessness, chest pain, unusual bleeding, very little urine, severe rash or blistering, or vomiting that stops you keeping fluids down. Use your unit's temperature threshold and emergency instructions. Do not wait until the next visit.

Between-treatment notes

Day 1: Day 1 treatment

Gemcitabine and cisplatin after blood tests and review. Confirm symptoms, blood results and support plan.

Day 2–7: At home

Follow the written support plan. Report new symptoms promptly.

Day 8: Day 8 treatment

Gemcitabine alone if blood tests and review allow. Do not assume Day 1 clearance covers Day 8.

Day 9–21: Recovery and review

Effects vary; no guaranteed recovery day. Keep the contact plan accessible.

Support around the medicines

1 Keep your systemic anticancer therapy (SACT) card with you

Show your treatment card to any healthcare professional. Keep daytime and out-of-hours oncology numbers accessible.

Side effects and action

Effects differ between people; report new symptoms early.

During a drip

Tell the nurse immediately about rash, flushing, chills, breathlessness, chest discomfort, faintness or pain/swelling around the cannula.

Blood and organ checks

Tests can detect changes before you notice symptoms. Normal results at the last visit do not rule out a problem now.

Everyday life

Plan support around how you feel.

Eating and drinking

Ask for help with nausea, appetite and mouth soreness. Follow your own fluid plan and tell the team if you cannot drink.

Medicines and fertility

Bring a full list of medicines and supplements. Discuss pregnancy prevention, breastfeeding and fertility before treatment.

Activity and help

Balance gentle activity with rest. Ask your team for help with symptoms, transport, work and caring responsibilities.

Checking progress

The treatment plan is reviewed throughout the course.

Scans and symptoms

Your consultant chooses scan timing according to the cancer and treatment plan. Blood tests primarily check safety and do not by themselves show whether treatment is working.

After chemotherapy

Agree the next treatment or follow-up plan, ongoing symptom support and whom to contact. Some nerve or hearing effects can persist.

Contacts and questions

Before leaving the clinic

Ask the team to complete your treatment aim, planned cycles, contact numbers and next appointment. Keep this summary with your SACT card.

Check your understanding

In your own words: which medicines are given on Day 1 and Day 8, which symptoms need a call today, and which number will you use out of hours?

Questions worth asking

How often do I attend?

Usually Day 1 for both medicines and Day 8 for gemcitabine in each 21-day cycle. Your unit confirms actual appointments and blood tests.

Does a delayed dose mean treatment failed?

No. Blood counts, kidney results or symptoms can make a delay or dose change safer. Your team decides whether and when to give the next dose.

Who do I call if I feel unwell?

Use the oncology number on your treatment summary or SACT card. Ask for a daytime and out-of-hours number before leaving. Do not use the QR website as an emergency contact service.

Try explaining the plan back: Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · gemcitabine / cisplatin

Active medicines: gemcitabine + cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before systemic anticancer therapy (SACT) on Day 1, then daily on Days 2–3

Medicine	Dose / route	When
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	General high-risk NCIS V 8 row Reference course only: reconcile the aprepitant interaction and any other steroids into one clear prescription.
Olanzapine	5 mg orally once daily	Day 1 and Days 2–4 General high-risk NCIS reference; review sedation, falls, metabolic risk and interaction with metoclopramide. Consultant may individualise.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/pharmacy must specify duration and suitability. Co-prescribing with olanzapine increases extrapyramidal risk; assess neurological contraindications and interactions.

Additional phase-specific support

- Follow one written anti-sickness prescription, including any tablets for home and rescue treatment. Day 8 gemcitabine has a separate lower-risk anti-sickness plan. Cisplatin needs prescribed intravenous (IV) hydration, electrolyte replacement and kidney, hearing and nerve checks. Follow your own fluid advice; tell the team if you have heart failure or a fluid restriction.
- Follow one reconciled prescription; the clinical support sheet is a reference, not an instruction to self-prescribe.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 · gemcitabine alone

Active medicines: gemcitabine

Emetogenic risk: low

Medicine	Dose / route	When
Dexamethasone	8 mg orally	Day 8 before gemcitabine if prescribed NCIS low-risk reference; individualise and reconcile with any other steroid course.

Additional phase-specific support

- Follow one written anti-sickness prescription, including any tablets for home and rescue treatment. Day 8 gemcitabine has a separate lower-risk anti-sickness plan. Cisplatin needs prescribed intravenous (IV) hydration, electrolyte replacement and kidney, hearing and nerve checks. Follow your own fluid advice; tell the team if you have heart failure or a fluid restriction.
- Follow one reconciled prescription; the clinical support sheet is a reference, not an instruction to self-prescribe.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Your anatomical side-effect map

Gemcitabine and cisplatin · lung cancer

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Breathing

Gemcitabine

Call urgently for new cough or breathlessness.

2 Mouth

Chemotherapy

Report painful sores or difficulty eating and drinking.

3 Liver

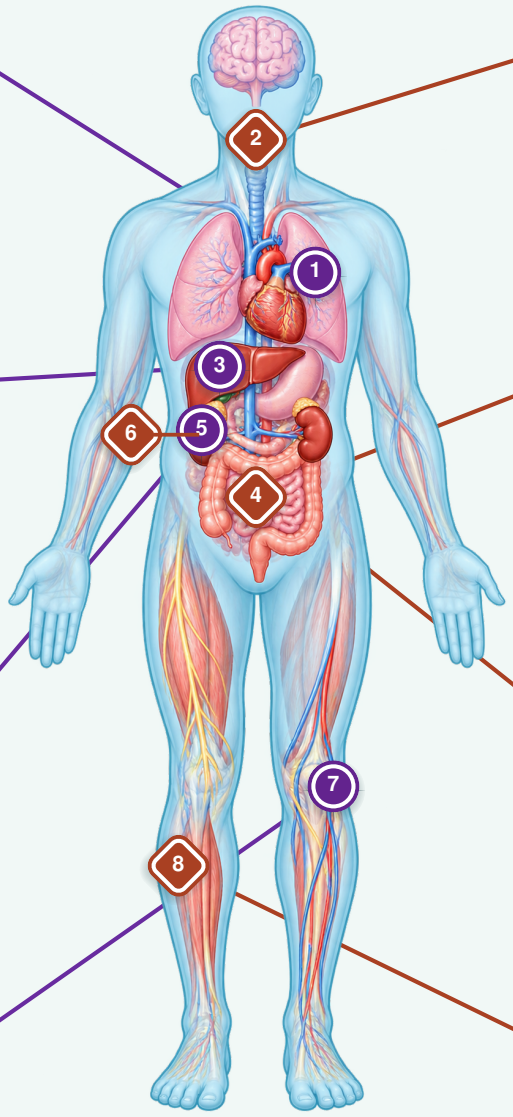
Gemcitabine

Blood tests check liver function; report yellow eyes or dark urine.

4 Sickness / bowels

Chemotherapy

Follow the anti-sickness plan; call if unable to keep fluids down.



5 Kidneys / urine

Cisplatin / gemcitabine

Report reduced urine or new swelling; keep kidney blood tests.

6 Blood / infection

Chemotherapy

Fever, sudden illness or unusual bleeding needs urgent advice.

7 Skin

Gemcitabine

Report widespread rash, blistering or peeling urgently.

8 Nerves / hearing

Cisplatin

Report numbness, tingling, ringing ears or hearing changes.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

SACTCHECK · CHAPTER 04

Your record and contact card

Gemcitabine and cisplatin · lung cancer · National Cancer Control Programme (NCCP) 00281

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Record only medicines actually given. Day 8 is gemcitabine alone; each dose needs separate clearance.

Reusable cycle record · Days 1 and 8 if prescribed · copy this sheet for another cycle of the same phase.

Gemcitabine · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999

Cisplatin · Platinum chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Hearing changes	□ No change	□ New ringing or muffled hearing: report promptly	□ Conversation difficult: contact the team	□ Sudden hearing loss: urgent assessment	□ Sudden weakness or collapse: 112/999
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Bleeding / bruising	□ None	□ New small bruise / brief bleed: report	□ Repeated bleeding: call today	□ Bleeding that will not stop: urgent help	□ Heavy bleeding / collapse: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

Gemcitabine and cisplatin · lung cancer

NCCP 00281 v7 · 21-day cycle

A cycle lasts 21 days (3 weeks). Day 1: gemcitabine and cisplatin. Day 8: gemcitabine alone after separate blood tests and review. Days 9–21: no scheduled chemotherapy. The usual planned course is 4–6 cycles, with earlier changes for progression or unacceptable effects.

1	2	3	4	5	6

Your contact plan

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
Emergency: 112 / 999.

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00281/ · no patient information.

Draft for review · SACTCheck v0.84.28

SACTCHECK · CHAPTER 05

Sources and further information

Gemcitabine and cisplatin · lung cancer · National Cancer Control Programme (NCCP) 00281

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00281 v7 · official regimen**
   https://healthservice.hse.ie/documents/6926/281_V7_GemCis75.pdf
2. **Zatloukal et al. 2003 · advanced non-small-cell lung cancer (NSCLC) randomized trial**
   https://pubmed.ncbi.nlm.nih.gov/12928123/
3. **NCCP NCIS antiemetics V 8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf